BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed magnetic resonance imaging (MRI) and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

Check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver	full blood count (FBC), renal/liver profile, glucose, hepatitis B surface antigen (HBsAg) and antibody to hepatitis B core antigen (anti-HBc)	FBC weekly during three-week radiotherapy (RT)	Continue FBC weekly for four weeks after RT; renal/liver function tests (LFT) at 1 and 4 weeks after RT	Day 1 FBC/renal/LFT; cycle 1 can use four-week post-RT review. EMA Day 22 FBC	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00461 v3 · current regimen · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Temozolomide with short-course radiotherapy, then adjuvant treatment: treatment and checks

Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461.

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00461	FBC, renal/liver profile, glucose, HBsAg and anti-HBc	FBC weekly during three-week RT	Continue FBC weekly for four weeks after RT; renal/LFT at 1 and 4 weeks after RT	Day 1 FBC/renal/LFT; cycle 1 can use four-week post-RT review. EMA Day 22 FBC	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function NCCP 00461	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00461	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1 Temozolomide with three weeks of radiotherapy

Daily capsule phase

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

2 Four-week break before adjuvant treatment

Recovery and review

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

3

Up to six adjuvant 28-day cycles

Five capsule days per cycle

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / **act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38